

Patient Name	:	SANGEETA 35/F
Age / Sex	:	25Y / Female
Date & Time	:	2026-10-02 19:45
Referring Doctor	:	
Investigation	:	MRI LUMBO-SACRAL SPINE

M.R.I OF LUMBO-SACRAL SPINE

Clinical History:

Sequences:

Multiplanar, multisequence MRI of the lumbosacral spine was performed, including sagittal and axial T1W, T2W and STIR sequences.

Study reveals:

Alignment:

Lumbar lordosis is maintained. No significant spondylolisthesis or scoliosis.

Vertebral Bodies:

Vertebral body heights are maintained. No focal marrow-replacing lesion or acute compression deformity identified.

Intervertebral Discs:

Mild degenerative disc changes at the lower lumbar level.

L1-L2:

No significant disc bulge or focal disc herniation. No significant spinal canal or neural foraminal stenosis.

L2-L3:

No significant disc bulge or focal disc herniation. No significant spinal canal or neural foraminal stenosis.

L3-L4:

No significant disc bulge or focal disc herniation. No significant spinal canal or neural foraminal stenosis.

L4-L5:

Mild diffuse circumferential disc bulge with associated ligamentum flavum hypertrophy, causing indentation of the ventral aspect of the thecal sac. There is narrowing of the bilateral lateral recesses and neural foramina, with abutment of the traversing and exiting nerve roots bilaterally. No definite nerve root compression identified.

L5-S1:

No significant disc bulge or focal disc herniation. No significant spinal canal or neural foraminal stenosis.

Central Canal:

No significant central canal stenosis at the remaining visualized levels.

Facet Joints:

No significant facet arthropathy identified on the provided findings.

Conus Medullaris:

Conus terminates at an expected level and demonstrates normal signal intensity.

Cauda Equina:

No gross abnormality of the visualized cauda equina nerve roots.

Paraspinal Soft Tissues:

No significant abnormality identified.

IMPRESSION:

- 1. Mild diffuse disc bulge at L4-L5 with ligamentum flavum hypertrophy, resulting in ventral thecal sac indentation and bilateral lateral recess and neural foraminal narrowing.
- 2. The disc bulge abuts the bilateral traversing and exiting nerve roots at L4-L5.
- 3. Mild lower lumbar degenerative spondylotic changes.

Clinical Advice:

Clinical correlation for further evaluation.

Signature



Dr. Varun Kumar
DMC No. : 19138
MD